AMERICAN PSYCHOLOGICAL ASSOCIATION SERVICES, INC.

June 16, 2025

Dr. Mehmet Oz, Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

Re: [CMS–0042–NC] Request for Information: Health Technology Ecosystem

Dear Administrator Oz:

On behalf of the American Psychological Association Services, Inc. (APA Services), please accept the following response to the Centers for Medicare and Medicaid Services' (CMS) Request for Information; Health Technology Ecosystem (CMS-0042-NC). APA Services is the companion organization of the American Psychological Association (APA), which is the largest scientific and professional organization representing the discipline and profession of psychology, as well as over 173,000 members and affiliates who are clinicians, researchers, educators, consultants, and students in psychological science.

APA Services appreciates the continued efforts of CMS to seek ways to implement effective and responsible adoption of technology to empower patients to make better decisions for their health and well-being. We support the administration's vision to invest in building a digital ecosystem to improve population health by creating efficiencies in care delivery, expanding access to care and health information, supporting prevention and early intervention services, empowering individuals, and improving the management of chronic disease management.

We also appreciate the opportunity to provide comments responding to specific questions focused on encouraging provider adoption of digital health technologies, evaluating impact of innovation on clinician workflows and considering interoperability/information blocking concerns. Fundamental for many psychologists and other mental/behavioral health and substance use disorder providers (MH/BH/SUD providers) is the fact that they work in small and solo practices lacking ample financial and staff resources to invest in the various technologies being considered in the RFI. Many of the questions raised in this RFI are premised on the assumption that most providers have already invested in health technologies like certified electronic health records (EHRs) offering sophisticated technical capabilities like online patient check-in or secure patient portal enabling patients to access their records and manage their information. Unless limitations on IT staff and financial resources are addressed, psychologists and other MH/BH/SUD providers will continue to face obstacles to implementing new health information technologies.

As discussed in further detail below, APA Services offers the following recommendations for your consideration:

Digital Health Technologies:

- Expand the categories of eligible devices under the G0552 code for Digital Mental Health Treatment (DMHT) services
- Establish a national reimbursement rate for G0552
- Specify telehealth coverage for DMHT services
- Advise Medicare Administrative Contractors (MACs)to require consistent and transparent coverage for G0552 as well as timely and transparent process for device claims reviews
- Establish funding opportunities – such as grants or subsidies for training, hardware, and software acquisition to implement and maintain advanced digital tools – to facilitate greater provider adoption of certified health information technologies
- Create AI guidance promoting transparency and informed consent for users, mitigating bias and misinformation, and promoting data confidentiality and security
- Encourage developers to create scalable, cost-effective health information technologies geared for small and rural health care practices to facilitate meaningful participation in state health information exchanges (HIEs)
- Coordinate with states to ensure greater uniformity in state HIEs and provider participation

Provider Workflows:

- Engage with the MH/BH/SUD treatment provider community about data standardization elements for the US Core Data for Interoperability (USCDI) consistent with workflow and data privacy needs unique to MH/BH and SUD services
- Encourage technical solutions identifying certain health data elements requiring a specific kind of patient consent and segmenting data in compliance with current laws especially in integrated care settings
- Reform quality reporting measures and measure development
- Establish benchmarks using National Provider Identifiers (NPIs)

Information Blocking Issues:

- Reconsider Adopting the previously proposed Requestor Preferences Exception
- Provide gradual implementation of disincentives with a focus on education and compliance education
- Promote greater education efforts regarding the information blocking reporting portal

Encouraging Provider Adoption of Digital Health Apps and other Health Technologies [PR1 - PR4]

To encourage broader adoption of digital health products, including approved health apps, electronic health records (EHRs) and other health technologies, especially in rural and underserved areas, we encourage CMS and its partners to consider the following:

1. Digital Behavioral Health Treatment (DMHT) Improvement: While we commend CMS for finalizing the creation of new G codes for DMHT devices furnished incident to or integral to professional behavioral health services used in conjunction with ongoing behavioral health care treatment under a behavioral health treatment plan of care, we encourage CMS to remove unnecessary limitations on G0552 (DMHT device code). Importantly, economic studies have shown the potential for DMHT devices to decrease overall medical spending. There are a variety of DMHT devices currently available in the U.S. that are specific to a mental health disorder (e.g. major depressive disorder and PTSD) while others target physical conditions by addressing the mental health component (e.g. chronic pain, anxiety and irritable bowel syndrome), but only some of these digital therapeutics meet the specific requirements of the G0552 code. We encourage CMS to expand the G0552 code to include devices that treat physical conditions using psychological interventions and many others that do not meet current device classification requirements.

As we noted in our response to the CY 2025 proposed Medicare Physician Fee Schedule, we believe requiring clearance under 21 CFR 882.5801 is unduly narrow. This regulation, originally intended as a catch-all for computerized behavioral therapy devices, does not encompass the current diversity of products that could be used as DMHT devices. FDA has since established additional regulatory device classifications tailored to different device types or indications for use and their respective patient risks and regulatory needs, and the agency is likely to create new device categories tailored to new DMHT device types as technology and treatment modalities evolve.

We believe that other devices cleared by the FDA that have appropriate requirements for efficacy and safety, appropriate quality systems controls, and are appropriate for use as a part of a behavioral health therapy plan should be allowed. FDA has established additional regulatory device classifications for digital behavioral treatments, including: Computerized Behavioral Therapy for the Treatment of Fibromyalgia Symptoms (21 CFR 882.5804); Computerized Behavioral Therapy Device for Treating Symptoms of Gastrointestinal Conditions (21 CFR 876.5960); Digital Therapeutic Software for Attention Deficit Hyperactivity Disorder (21 CFR 882.5803); and Digital Therapy Device to Reduce Sleep Disturbance for Psychiatric Conditions (e.g., nightmare disorder, PTSD, etc.) (21 CFR 882.5705). There may be additional regulatory device classifications issued for other digital behavioral therapies, which should also qualify for payment under these new codes as new products come to market.

We suggest that any digital cognitive behavioral therapy that is cleared or approved under section 510(k), 513(f)(2), or 515 of the Federal Food, Drug, and Cosmetic Act be eligible for payment under these codes if they meet the following criteria:

- Involve cognitive behavioral therapy or are otherwise treatment for behavioral health conditions;
- Include special controls from FDA requiring clinical evidence to demonstrate safety & efficacy; and
- Do not include a hardware device (other than a patient's own phone or computer).

We recommend CMS reconsider limiting G0552 to only devices falling under 21 CFR 882.5801. Specifically, we request that devices cleared under regulations (21 CFR 882.5804), (21 CFR 876.5960); (21 CFR 882.5803); and (21 CFR 882.5705) also be included for payment under G0552.

We were also disappointed that in the final rule CMS did not set a reimbursement rate for G0552, resulting in carrier pricing. As we stated in our comment letter, we believe there are adequate data sources to support national rate setting for G0552 and that a national rate will facilitate access and utilization. We encourage CMS to reconsider establishing a national rate for G0552. The new DMHT code discussed in this request is an inherently remote service — the patient receives access to the DMHT from their practitioner for use in the patient's home. Although the descriptor for G0552 does not mention the service's location, it was intended to include both virtual/remote and in-person services. Given the manner in which the diagnosis and on-going psychotherapy services are currently furnished, it was never contemplated that G0552 would only be furnished in person.

We urge CMS to clarify that G0552 can be furnished via telehealth or add the code to the telehealth list to ensure that practitioners who are permitted to offer DMHTs to appropriate patients may do so remotely.

Finally, we urge CMS to issue guidance to Medicare Administrative Contractors (MACs) requiring coverage of new DMHT codes and instructing MACs to develop a timely and transparent process for G0552 device claims reviews. The delay by MACs in establishing consistent and transparent reimbursement for G0552 is limiting access to these new devices and resulting in inconsistent coverage of effective behavioral health treatments for Medicare beneficiaries.

2. Expand Funding Opportunities for EHR Adoption: Many MH/BH providers were excluded from financial incentives for adoption of EHR systems and other digital tools offered under the Health Information Technology for Economic and Clinical Health Act of 2009 (HITECH Act) (Pub. L. 111-5) and continue to face financial barriers to adopting these critical resources. CMS should create targeted funding programs for these providers, particularly those in rural or independent practice settings.

A significant obstacle that has contributed to the slow adoption of EHR systems by MH/BH providers is the exclusion of MH/BH/SUD providers from the definition of "meaningful user" under the HITECH Act. As a result, those providers have lacked access to critical incentive funding to cover the significant start-up costs for acquiring and maintaining a robust and secure electronic health records system. Since the meaningful use program focused on physicians and hospitals, the design and acquisition of many EHR systems (especially for larger, integrated care systems) have often not contemplated inclusion of MH/BH/SUD data, which is subject to greater privacy protection compared to other health data.

Since that time, psychologists, and other MH/BH providers, especially those in small and independent practices, have fallen far behind physicians, medical health care practices and large health care systems in terms of their ability to exchange electronic health data in a secure manner. As the federal government's agenda to promote interoperability and greater patient access to their own data continues to advance, that gap between the haves and the have-nots is glaring. The price for MH/BH providers to try to catch up and close the gap is high and for some, even cost prohibitive. Investing in the software, hardware and training required to use an EHR system is expensive. BH/MH providers generally have low operating margins and limited working capital available to invest in EHR adoption.

Without some sort of funding support, the gaps in EHR adoption will persist, making the goals of health care data integration, secure data exchange across systems and increased patient access to their data difficult to achieve. Therefore, we urge CMS to endorse funding opportunities – such as grants or subsidies for training, hardware, and software acquisition to implement and maintain advanced digital tools -- for providers who were ineligible for incentive payments under the HITECH Act.

3. Concerns about AI and Algorithmic Tools: The integration of artificial intelligence (AI) into behavioral health is evolving rapidly, presenting opportunities and challenges for MH/BH providers. AI can enhance clinical decision-making and outcomes, improve access to care, and enhance provider workflow and efficiency. As AI tools become increasingly integrated into healthcare and other workforce settings, human oversight and thoughtful human-technology interaction are critical. However, in its current state, significant ethical issues exist, including transparency, bias, data security, and misinformation. In addition, there are issues of professional liability.

- **Transparency and Informed Consent**: There is a need for transparency in how algorithms process and store sensitive mental health information. AI use in MH/BH settings should be disclosed to other relevant providers, individuals who are receiving direct care services, and any third parties who may be considered a client of services (e.g., the court) in a culturally and linguistically appropriate manner.

- **Mitigating Bias**: AI systems should be evaluated with a focus on addressing bias and preventing exacerbation of existing healthcare disparities. Responsible AI development considers the full range of lived experiences to avoid unfair discrimination. To ensure AI serves everyone equitably, MH/BH providers should play a role in helping to build high-quality, representative datasets, especially from underrepresented regions, which are consistent with data privacy requirements.

- **Data Privacy and Security**: AI systems handling sensitive behavioral health data pose risks related to privacy breaches and unethical data use. Strong cybersecurity measures should be in place to prevent data breaches or misuse such as compliance with HIPAA and other relevant data privacy regulations.

- **Accuracy and Misinformation Risks**: AI tools should be rigorously validated before implementation in mental and behavioral health care settings. Products and services that rely on AI models should undergo rigorous accuracy testing to ensure reliability and this information should be easily identified by the developers (e.g., the model developers have published their testing/reliability data and outcomes).

- **Human Oversight**: AI should augment, not replace, human decision-making. This can be accomplished by establishing clear human intervention points (i.e., human in the loop) within AI-driven workflows to maintain professional accountability and to ensure that AI-generated insights are reviewed critically.

4. Support Scalable, Interoperable Solutions and Technical Assistance for Rural, Solo and Small Practices: Encourage health information technology vendors to develop certified EHR systems tailored to small-scale MH/BH practices that meet interoperability standards while addressing privacy and usability concerns. State HIEs rely on providers' EHR systems to be interoperable to facilitate meaningful exchange

of certain health data. Without scalable certified EHR systems geared for rural and small practices, many MH/BH providers are unable to participate in state HIEs.

5. Coordinate with States: Work with state governments to align incentives and technical assistance programs that promote digital health adoption among providers not traditionally engaged in federal programs. This includes encouraging states to develop more standardization and consistency across their respective HIEs in terms of what kinds of providers are mandated to participate, what are the minimum necessary categories of data required for the HIE to be beneficial, what alternatives are there for providers who lack certified EHRs to connect to HIEs, and how to allow patients to opt-in versus opt-out of the HIE.

Obstacles to Innovation in MH/BH Provider Workflows [PR5-PR8]

Several barriers hinder the development and effective use of digital tools for quality reporting, documentation, and billing:

1. Lack of MH/BH Data Standardization: MH/BH data lacks standardized formats, making integration into EHRs and reporting systems difficult. CMS should work with ONC to define standardized MH/BH data elements within the US Core Data for Interoperability (USCDI).

For the purposes of interoperability, there continues to be a lack of clarity as to what kinds of MH/BH patient data ought to be shared and how that data is recorded or characterized. Many psychologists and other MH/BH providers work in small or solo practices where integrated and accessible patient data is not a concern. So, it is not surprising that there is significant discretion and subjectivity among MH/BH providers as to how much detail is needed in patients' clinical files beyond the standard data fields like patient identifiers, diagnosis, treatment plan and dates of service. In other words, some MH/BH providers may record a lot of narrative details about what is discussed in their session with patients, keeping the private details maintained in separate psychotherapy notes whereas more objective data is included in the patient's clinical record (consistent with HIPAA requirements). Others might keep more concise, basic notes.

Clinical diagnosis might be recorded in specific or general terms (e.g., PTSD/trauma versus generalized anxiety). The use of diagnostic coding might vary across providers. For screenings, assessments and testing, there may be variability across providers as to how raw test data is recorded and stored within the patient's file. For example, if the psychological or neuropsychological test requires the test taker to record responses on the hard copy of the test instrument, then the raw test data, which *might* be accessible by the patient, would be commingled with the test instrument which is subject to copyright protection. Until more psychological and neuropsychological tests are normed for digital administration, formatted to keep copyrighted materials separate from test responses and made compatible with EHR systems, MH/BH providers who use these tests will have a difficult time adopting EHR technology since MH/BH providers' recordkeeping needs do not neatly fit within many existing EHR frameworks. This is also compounded by the lack of understanding by many other kinds of providers, health care administrators, and payors about their entitlement to test data beyond the provider's test report.

As a result, a challenge exists in identifying what exact kinds of MH/BH data are needed for meaningful health data exchange to facilitate better care coordination and administrative efficiencies and how to standardize those data categories for use across systems. The US Core Data for Interoperability (USCDI)

lists the required standardized data classes and elements that health IT developers need to include to facilitate interoperable health data exchange. However, a review of the USCDI iterations reveals that the focus has been primarily on physical health data without any specific references to MH/BH data. If developers and certified HIEs are using the USCDI for health information exchange, further consideration needs to be given to identifying exactly what MH/BH data ought to be included for meaningful use and how each data element ought to be defined. From a technical standpoint, it would also be helpful to consider whether current systems generally capture medical data through prepopulated dropdown menus versus narrative data fields and whether that functionality is feasible for MH/BH data.

Without clearer guidance on what exact data elements are specifically included for MH/BH data categories under the USCDI, it is nearly impossible for MH/BH providers to know how to comply with participation requirements for state-based HIEs and interoperable health data exchanges generally as states tend to follow federal guidance. We urge CMS and ATSP/ONC to consider the unique needs and obligations of MH/BH providers in determining what kinds of standardized MH/BH data elements are required for meaningful data exchange to ensure psychologists and other MH/BH providers can continue to comply with existing patient confidentiality and professional ethical obligations.

2. Privacy and Consent Complexity: Varying state laws on patient consent, patient confidentiality and data privacy can create compliance challenges. HIPAA allows states to impose stricter patient privacy laws, including patient consent requirements beyond what is required by the Privacy Rule. In addition, HIPAA's heightened protection for psychotherapy notes creates complexity, which is discussed in the next section. Regarding patient consent requirements for electronic health data, there may be state laws regarding electronic disclosure of personal data and special rules regarding specific types of health data. For mental health records, there may be additional or stricter consent requirements.

APA fully supports states having stronger privacy protections. But to create a successful data sharing framework, it will be critical to develop a data governance structure that incorporates how certain data will be shared in compliance with existing and varying state privacy and security rules. For that reason, we think that it is important that CMS and ASTP/ONC encourage developers to design and deploy technical solutions that would create an alert when certain health data elements require a specific kind of patient consent and segment data subject to stricter privacy requirements. That kind of functionality would benefit MH/BH providers who have concerns about compliance with federal and state privacy laws as well as other providers who might not appreciate the additional privacy requirements and patients who want to ensure that certain sensitive data remains confidential. Not only could these added functionalities, if feasible, generate greater confidence in these systems, but it may also spur higher rates of adoption and increased HIE participation. We support CMS and its partners promoting technical solutions to balance privacy protections for MH/BH data with secure data exchange capabilities.

3. Limited Data Segmentation for Integrated Care Settings: Many EHR vendors do not offer features like data segmentation or support for MH/BH workflows. CMS should incentivize vendors to develop certified systems that meet the unique needs of MH/BH and SUD providers.

While health information technologies (like EHRs) can improve efficiencies and increase access to patient data, this capability needs to be balanced against federal and state privacy laws granting greater protection to very sensitive patient data (typically MH/BH and SUD data) that should be protected from disclosure to third parties (including, but not limited to, payers and other providers) and to patients

themselves. Compared to a patient's other clinical data, HIPAA gives greater protection to psychotherapy notes so long as those notes are kept separately from the rest of the patient's clinical record. Therefore, not all patient data maintained by MH/BH providers would be subject to the broader patient access and information-sharing associated with EHR use.

For MH/BH providers who keep psychotherapy notes, it is critical that those notes retain their higher level of protection. Therefore, including some sort of data segmentation for psychotherapy notes from the rest of a patient's health record within EHR systems is necessary to ensure the additional protections under HIPAA are followed. In addition to psychotherapy notes, other sensitive health data including specific information concerning an individual's mental health may be subject to certain state laws mandating heightened protections and consent documentation requirements for disclosure or redisclosure, which would also necessitate additional segmentation requirements.

If MH/BH providers are unable to provide input in the design and setup of an EHR system, consideration of data segmentation may be overlooked and therefore not contemplated in the system design. This leaves many MH/BH providers struggling with merging their recordkeeping processes with an EHR system, especially in integrated care settings, and it would be difficult to retrofit existing proprietary EHRs to accommodate some data segmentation capabilities needed for MH/BH providers. Therefore, it is important that CMS and ATSP/ONC encourage developers and vendors to include data segmentation capabilities in designing certified EHR systems. Alternatively, more developers and vendors should consider developing certified EHR systems that are designed specifically for MH/BH providers but allow secure data exchange for specific limited data sets supporting coordinated care.

Many MH/BH providers operate in smaller private practice settings and therefore, need health information solutions that achieve the goals of interoperability and security on a smaller scale. Currently, many certified health IT products are designed for larger, complex health care sites but there is a market for MH/BH providers who need interoperable, scalable options especially in those jurisdictions that are mandating MH/BH providers to participate in the state HIE.

4. Clinical Quality Data Reporting Simplification: Several aspects of Medicare's quality and data reporting requirements, particularly within the Merit-based incentive Payment System (MIPS), impose significant and often counterproductive administrative burdens on our members. These burdens undermine efficient practice and can disincentivize meaningful participation in quality improvement. We recommend the following changes to support psychologists participating in Medicare:

- **Maintain MIPS Reporting Option:** MIPS Value Pathways (MVPs), which are core measure sets, may address quality payment reporting burden but are only likely to be feasibly implemented for certain settings, such as long-term care, or for certain subspecialty provider populations, such as neuropsychologists. MVPs are not likely to be feasibly implemented for psychologists who have generalist practices and see between 30 – 40 patients per week who present with varying diagnoses (e.g., depression, anxiety, PTSD, psychosis, etc.) and represent patients across the lifespan. No single core measure set, or MVP, could account for the level of patient heterogeneity a generalist psychologist provides treatment to, and having to implement multiple different MVPs would increase the burden that ECs currently experience with traditional MIPS. One solution would be to retain traditional MIPS for those providers for whom no MVPs are relevant or are not feasibly able to be implemented and instead reduce the number of quality

measures reported from 6 to 4 to better align with MVPs, and reduce burden while giving providers adequate options in selecting the measures that are most meaningful to their particular patient population.

- **Provide increased flexibility in Qualified Clinical Data Registry (QCDR) measure development:** MIPS reporting can be challenging, especially finding quality measures that are relevant to specific specialties, such as mental and behavioral health. This is, in part, why CMS established qualified clinical data registries (QCDRs) as a way for specialty societies who did not have access to enough quality measures to develop these measures for their members with more flexible development requirements than going through the CMS measures under consideration process. However, over the last several years, CMS has made measure development for QCDRs overly onerous, constantly changing testing requirements with little direction on those requirements. Further, they have continued to discourage the development and approval of process measures in lieu of favoring outcome measures. APA has prioritized the development and implementation of patient-reported outcomes measures (PROS). However, it has also become clear that there is a clinical need for additional process measures, which are particularly critical to neuropsychologists and providers who conduct psychological assessments as these care models typically involve only one or two meetings with the Medicare beneficiary, with no ongoing relationship. Unfortunately, it has been particularly challenging to get QCDR process measures approved. We recommend that CMS apply flexibility to programmatic requirements related to QCDR measure development and approve new process measures within QCDRs when appropriately supported by a strong clinical rationale.

- **Establish benchmarks based on National Provider Identifiers (NPIs):** Benchmarks are currently established by calculating the number of provider Taxpayer Identification Numbers (TINs), as opposed to the number of National Provider Identifiers (NPIs), having reported on a measure. This approach disenfranchises specialties that have small numbers of required reporters, making it impossible to get most QCDR quality measures benchmarked, regardless of how relevant and meaningful they are to that provider and patient population. The unintended consequence of this approach is that eligible clinicians (EC's) are strongly disincentivized from reporting on specialty measures and instead choose measures they know they can score enough points on to avoid a payment penalty. We recommend establishing quality measure benchmarks by calculating the number of National Provider Identifiers (NPIs) as opposed to Taxpayer Identification Numbers (TINs), having reported on a measure, which would reduce burdens on providers by allowing them to access more meaningful measures and meeting scoring requirements without the fear of a negative payment adjustment.

Information Blocking Exceptions and Disincentives [PR12-PR14]

1. Revising Information Blocking Exceptions: APA Services previously provided comments on the HTI-3 proposed rulemaking, supporting the proposed revisions to the information blocking exceptions under 45 CFR part 171 in response to real-world challenges faced by providers, particularly those in behavioral and mental health care. Following the release of the HTI-3 Final Rule in late 2024, we offer the updated recommendation:

- **Reconsider Adoption of Requestor Preferences Exception**: We had previously commented in support of a new proposed exception – Requestor Preferences Exception -- wherein the psychologist would not be engaging in information blocking if the requestor indicates verbally or in writing that there be limitations on a subset of EHI, including under what circumstances and timing that the EHI can be made available to the requestor. This exception would provide a helpful opportunity for health care providers and patients alike to allow for a temporary hold on the release of certain EHI under certain circumstances. One such example involves the patient choosing to delay accessing certain EHI – such as test results – until either the provider has reviewed the results, or the patient meets with the provider to review and discuss the results together. This exception would also be helpful in circumstances involving provider-to-provider communications where a provider may request only a specific subset of a patient's EHI instead of the entire patient record being shared. Likewise, the provider would need to honor any patient request to remove or suspend the restriction. We recognize that the limited circumstances permitting such a request would need to be clearly communicated and documented to ensure that the patient was not subject to any undue encouragement or inducement and that the provider was not intentionally engaging in information blocking. We hope that this proposed exception might be reconsidered in future rulemaking.

2. Effective Disincentives for Information Blocking: For provider categories not currently subject to CMS-established disincentives, APA Services recommends:

- **Graduated Enforcement Mechanisms**: Implement a tiered approach to disincentives, starting with education and technical assistance, followed by financial penalties or reduced eligibility for federal funding for repeat or egregious violations.
- **Link to Certification and Participation**: Tie compliance with information blocking rules to eligibility for participation in federal programs or receipt of ONC-certified EHR incentives, especially for providers who were previously excluded from HITECH incentives.
- **Transparency and Reporting**: Consider public reporting of confirmed violations to promote accountability and encourage voluntary compliance.

3. Provider Engagement with the Information Blocking Portal: Many MH/BH providers may be unaware that this portal exists or unsure of what constitutes a reportable offense. We encourage CMS to develop targeted educational materials and webinars to raise awareness, particularly among small and independent practices. This may include providing clear examples of information blocking scenarios that providers in solo and small practices may encounter. Many providers may be unclear as to what the consequences may be or how complaints are handled.

- **Simplify the Reporting Process**: Streamline the portal interface and provide clear examples of information blocking scenarios. Consider allowing anonymous or confidential submissions to reduce fear of retaliation.
- **Clarify Protections and Outcomes**: Providers may be hesitant to report due to uncertainty about the consequences. CMS should clarify what protections are in place for whistleblowers and how complaints are handled.
- **Impact**: Encouraging complaint submissions would improve enforcement and help identify systemic barriers to interoperability. However, it is essential that this process not become punitive or burdensome, especially for providers already navigating complex privacy and

compliance environments. A balanced approach that emphasizes education, support, and transparency will be most effective.

Conclusion

APA Services thanks CMS for this opportunity to provide feedback on the issues and questions presented in the Health Technology Ecosystem RFI. We look forward to working with CMS and the administration to develop policies to promote the development of a health technology ecosystem that will improve population health. If your staff have any questions, you are welcome to contact Stephen Gillaspy, Deputy Chief for Health Policy & Health Care Financing at SGillaspy@apa.org.

Cordially,

Katherine B. McGuire, MS
Chief Advocacy Officer
American Psychological Association Services, Inc.